

Ponatinib / Asciminib nella LMC – Newsletter interna (ultimi 15 giorni)

Generata il 01/05/2026

Generata il: 2026-05-01

Newsletter interna – sintesi operativa

Messaggi chiave (da leggere in 60 secondi):

- Nel periodo emergono soprattutto temi su **Dose strategy / consolidamento** (n=4).
- Secondo filone: **Safety e comorbidità** (n=4).
- Terzo filone: **TFR / stop terapia** (n=3).

Cosa cambia per noi (azioni pratiche):

- Rivedere/rafforzare **monitoraggio cardiovascolare e interazioni** (es. anticoagulanti) nei pazienti in TKI.
- Valutare criteri interni per **consolidamento / de-escalation / TFR** nei pazienti in deep response.
- Verificare percorso di **test mutazionale** e criteri di switch nelle resistenze (incl. T315I).

Safety radar (segnali/interazioni da monitorare):

- Monitorare evoluzione su **TFR / stop terapia** (nuovi segnali/dati).
 - Monitorare evoluzione su **Safety e comorbidità** (nuovi segnali/dati).
 - Monitorare evoluzione su **Resistenza / T315I / mutazioni** (nuovi segnali/dati).
-

Top 5 da leggere subito (ordinati per impatto)

1) Cardiovascular Events and Management Strategies Including Tyrosine Kinase Inhibitor Switching, Dose Reduction, and Discontinuation in Chronic Myeloid Leukemia: A Single-Center Retrospective Study.

Altro

Cureus

Score: 13

Focus clinico: TFR / stop terapia, Dose strategy / consolidamento, Safety e comorbidità, Sequencing / linee di terapia

PMID: 42017086 | 10.7759/cureus.105578

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42017086/>

- **Tipo di studio/popolazione:** This study retrospectively analyzed the data on CVEs in 63 patients with chronic myeloid leukemia treated with TKIs at a single center between January 1, 2013, and October 31, 2023.
- **Risultato chiave:** TKI treatments were discontinued in one patient and 16 patients owing to clinically significant CVEs and adverse events other than CVEs, respectively.
- **Rilevanza clinica:** Switching to IM or BOS, TKI dose reduction, or treatment discontinuation was observed to be associated with improvement in cardiovascular parameters in selected patients achieving deep molecular response.

2) Asciminib in Philadelphia Chromosome-Positive Acute Lymphoblastic Leukemia: A Case Series and Review of Emerging Evidence.

Review

Hematology reports

Score: 11

Focus clinico: Resistenza / T315I / mutazioni, Dose strategy / consolidamento, Safety e comorbidità, Sequencing / linee di terapia, Real-world (utilità pratica), Sintesi (review/linee guida)

PMID: 42042193 | 10.3390/hematolrep18020028

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42042193/>

- **Tipo di studio/popolazione:** Asciminib, a first-in-class allosteric <i>BCR::ABL1</i> (STAMP) inhibitor, has demonstrated efficacy and favorable tolerability in chronic myeloid leukemia, but its optimal role in Ph+ ALL remains to be defined.
- **Risultato chiave:** Clinical outcomes are contextualized through a comprehensive review of emerging clinical trial data, real-world cohorts, and mechanistic studies evaluating asciminib in Ph+ ALL.
- **Rilevanza clinica:** Emerging structural biomarkers and ongoing clinical trials are expected to further refine patient selection, sequencing, and optimal integration of asciminib, particularly in CNS-involved disease and post-CAR-T cell relapse.

3) Asciminib Demonstrates Preserved Vascular and Endothelial Function Supporting a Favourable Cardiovascular Safety Profile.

Altro*Basic & clinical pharmacology & toxicology*

Score: 7

Focus clinico: Safety e comorbidità

PMID: 42010747 | 10.1111/bcpt.70240

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42010747/>

- **Tipo di studio/popolazione:** non riportato (abstract non disponibile).
- **Risultato chiave:** non riportato.
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4) Importance of five membered heterocyclic compounds in treatment of chronic myeloid leukemia by targeting various pathways.

Review*Future medicinal chemistry*

Score: 6

Focus clinico: Resistenza / T315I / mutazioni, Sintesi (review/linee guida)

PMID: 42011924 | 10.1080/17568919.2026.2658831

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- **Tipo di studio/popolazione:** Chronic myeloid leukemia (CML) is a clonal hematopoietic malignancy characterized by the presence of the Philadelphia chromosome, which generates the constitutively active BCR::ABL1 tyrosine kinase, driving uncontrolled myeloid proliferation.
- **Risultato chiave:** Although BCR::ABL1-targeted tyrosine kinase inhibitors (TKIs) have significantly improved CML management, challenges such as drug resistance, intolerance, and long-term adverse effects persist.
- **Rilevanza clinica:** Several clinically relevant agents, including dasatinib, ponatinib, and asciminib, incorporate such heterocyclic frameworks, highlighting their therapeutic significance.

5) BCR-ABL1 Drives Transcriptional Reprogramming of Chronic Myeloid Leukemia Cells for Immune Evasion Through C/EBP β .

Preclinico / laboratorio

MedComm

Score: 5

Focus clinico: TFR / stop terapia

PMID: 42051376 | 10.1002/mco2.70747

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- **Tipo di studio/popolazione:** Emerging immunotherapy holds promise to achieve treatment-free remission (TFR) for chronic myeloid leukemia (CML) patients, the development of which depends on full understanding of mechanisms driving immune evasion.
- **Risultato chiave:** In coculture, these BCR-ABL1 expressing neutrophil-like CML cells significantly inhibited T cell proliferation.
- **Rilevanza clinica:** Accordingly, combination treatment with targeted therapy using ponatinib and immunotherapy with anti-PD1 antibody not only provides rapid remission, but also delayed relapses after treatment discontinuation, justifying combination treatment for TFR of CML.

Evidenze per domande cliniche

Dose strategy / consolidamento (4)

Score: 0

Mini-riassunto (da abstract, estrattivo):

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Altro

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Mini-riassunto (da abstract, estrattivo):

Successful Management of Tyrosine Kinase Inhibitor-Induced Bone Marrow Aplasia in Chronic Phase Chronic Myeloid Leukemia with Ponatinib: A Case Report and Literature Review.

Review

Clinical medicine insights. Case reports

Score: 5

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- **Tipo di studio/popolazione:** A 35-year-old man with chronic-phase CML developed persistent bone marrow aplasia after sequential treatment with imatinib and dasatinib.
- **Risultato chiave:** In the absence of a matched donor and response (*BCR::ABL1*: 25% IS), ponatinib was initiated at 45 mg daily and later reduced to 15 mg.
- **Rilevanza clinica:** Remarkably, hematologic recovery occurred, with normalization of peripheral blood counts, without recurrence of aplasia, and the patient achieved a sustained complete cytogenetic response within 15 months (*BCR::ABL1* < 1% IS).

Mini-riassunto (da abstract, estrattivo):

ASC4OPT: asciminib treatment optimization study in patients with chronic myeloid leukemia in chronic phase previously treated with two or more tyrosine kinase inhibitors.

Altro

Leukemia

Score: 5

PMID: 42026180 | 10.1038/s41375-026-02965-8

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42026180/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** The ASC4OPT non-comparative phase 3b study (NCT04948333) evaluates asciminib once daily (QD) or twice daily (BID) in chronic myeloid leukemia in chronic phase (CML-CP) treated with ≥ 2 tyrosine kinase inhibitors (TKIs).
- **Risultato chiave:** This study enrolled 169 patients not in major molecular response (MMR), with unsatisfactory response (intolerant, warning or failure) as defined by European LeukemiaNet (ELN) 2020 criteria.
- **Rilevanza clinica:** Among 40 patients who had their asciminib dose escalated to 200 mg QD, 17.5% were in MMR at Week 96.

Safety e comorbidità (4)

Score: 0

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Mini-riassunto (da abstract, estrattivo):

Riassunto aggiuntivo (full text Open Access, estrattivo):

Ponatinib confers adult human cardiomyocyte toxicity via inhibition of AKT signaling.

Preclinico / laboratorio

Physiological reports

Score: 5

PMID: 41995108 | 10.14814/phy2.70877

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/41995108/>

Open Access (Europe PMC/PMC): sì (PMC13088335)

- **Tipo di studio/popolazione:** As a widely used anticancer drug for the treatment of chronic myeloid leukemia, ponatinib is known to cause severe cardiovascular toxicities.
- **Risultato chiave:** Cardiomyocytes are the major functional units in the myocardium, and impairment of their viability and function plays a crucial role in cardiotoxic responses.
- **Rilevanza clinica:** Treatment of hPCMs with AKT inhibitor MK-2206 phenocopied ponatinib, while restoration of AKT signaling with UCL-TRO-1938 or insulin partially rescued hPCMs from cell death, suggesting a potential protective effect.
- **Metodi (full text OA):** In this study, all hPCMs were isolated from left atrial appendages of patients undergoing coronary bypass surgery.
- **Risultato principale (full text OA):** To assess the cellular response of hPCMs to ponatinib, we performed a concentration■response experiment using 10 different doses of the drug.
- **Dettaglio utile (full text OA):** Consistent with staining results, both ponatinib and MK■2206 significantly reduced cell survival (Figure 3f).
- **Messaggio clinico (full text OA):** It has been reported that ponatinib induces paradoxical activation of the MAPK pathway via BRAF■CRAF dimerization, resulting in ERK1/2 activation in other model systems, which may explain this unexpected observation (Wilhelm et al., 2023).

TFR / stop terapia (3)

Score: 0

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Sintesi (review/linee guida) (3)

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Review

Future medicinal chemistry

Score: 6

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Resistenza / T315I / mutazioni (2)

Score: 0

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Sequencing / linee di terapia (2)

Score: 0

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Real-world (utilità pratica) (1)

Score: 0

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Altro (1)

Score: 0

Mini-riassunto (da abstract, estrattivo):

Targeting the BCR::ABL1 Kinase: Advances Beyond Imatinib in Chronic Myeloid Leukemia.

Altro

Current hematologic malignancy reports

Score: 3

PMID: 42033491 | 10.1007/s11899-026-00778-1

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42033491/>

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